

GODWINS FAMILY CARE · ONE PAGE OF TRUTH

Release Runway

Where to continue on OpenEMR and where to continue on the app. Replaces the master checklist, the remediation plan, and the 8.4 upgrade plan — those three are removed from the repo. Target: release-ready Tuesday 09/08.

Decision carried into this plan. Write capability for orders and the fee sheet is **kept, not traded away**. The upgrade to OpenEMR 8.4 plus a bounded route patch is the path to getting it. Deep-linking the clinician into OpenEMR is the fallback only if the patch does not land in time.

Where things stand

Reconciled against the repo on 2026-09-05. All Session 4 work is merged.

Item	State
Sessions 1 – 3.5	Merged. Portal, gated intake, consents, Transfer-of-Care ROI, staff enrollment view, offline onboarding, reconciliation.
Session 4.1 — clinician workspace	Merged, PR #19. Chart, H&P, med rec, care plan with signed PDF, enrollment checklist.
Session 4.2 — clinician scheduling	Merged, PR #22. OpenEMR-tied calendar, conflict rejection, encounter linkage.
Session 4.4 — clinical completeness	Merged, PR #29. Follow-up SOAP, ICD-10 diagnoses, CPT capture, Rx, orders, sign-and-close, coding queue, coding assist T1+T2.
4.1 orphaned-notes defect	Fixed and merged. Notes were writing to patient zero. Pre-4.4 test encounters must be re-documented before they can be signed.
Session 4.3 — patient clinical read	Prompt written, not built. POA acting gates and case-manager scoped read are in its scope.
OpenEMR	7.0.4 live at emr.godwinsfamilycarellc.com. Identity, facilities, codes, payers not yet configured.

Two setup values are still empty and block signing by design: the billing NPI, and each clinician's NPI on their user record.

Track A — OpenEMR

Everything here is yours or your EMR maintainer's. None of it needs the app team.

A1. Upgrade to 8.4 · Saturday–Sunday

Verified against 8.4 source: it adds **POST /api/prescription**, and it fixes both server bugs — the document-endpoint 500 (missing bind array) and the vitals 500 (null user id). The two local patches we planned are no longer needed.

Step	Detail
1. Snapshot	RDS snapshot of gfc-openemr-db, plus an EBS snapshot of the gfc-openemr volume. Both must read Completed before you continue.
2. Rehearse	Restore the snapshot to a scratch DB and a scratch EC2. Change the image tag from openemr/openemr:7.0.4 to 8.4.0, then <code>docker compose pull && docker compose up -d</code> . Watch the logs — six schema upgrades run in sequence. Do not interrupt.

Step	Detail
3. Verify scratch	Admin login, patient chart, Fee Sheet, Billing Manager, entered CPT codes, facilities, users. Note any menu paths that moved — 8.x differs from 7.x and both guides were written against 7.0.4.
4. Re-register OAuth	Scopes bind at registration. Register the app's client fresh against scratch, adding user/prescription.read, user/procedure.read, user/list.read, user/ValueSet.read, user/drug.read.
5. Production	Only after scratch passes. Same tag change, same pull and up. Then delete the scratch instances — they bill hourly.

A2. The orders and fee-sheet write patch · Sunday–Monday

This is the finding that keeps write capability. 8.4 still ships no API route for order writes or fee-sheet charges — but the business logic already exists and is what OpenEMR's own screens call. `BillingUtilities::addBilling()` is the exact function the Fee Sheet uses to insert a charge. `ProcedureService` already carries `addDiagnosis`, `updateOrderCode` and `deleteOrderCode`. The patch adds controller methods and route lines over logic that is already tested in production use — it does not write billing logic from scratch.

Scope of the patch, in order of value:

1. POST a fee-sheet charge for an encounter, wrapping `BillingUtilities::addBilling()`. Carries CPT, ICD pointers, units, modifiers, rendering provider.
2. POST a procedure order, plus the order-code rows. `ProcedureService` has update and delete already; the create path is what is missing.
3. Optional: a code-search route over the loaded ICD-10 and CPT tables, which removes the last app-side workaround.

Keep the patch narrow and record it (file, diff, date, version) so a later upgrade does not silently revert it. If it is clean, upstream it — OpenEMR is open source and these are gaps others hit too.

A3. Configuration · Monday

Do this after the upgrade, so you configure once on the version you will actually run. The authority is the Billing & Configuration Guide v3.0 in the repo; where it disagrees with the older deploy guide, v3.0 wins.

Area	What to do	Watch for
Unblock the app	Four ACL grants, one screen — see A4 below. Then load the ICD-10-CM code set (Administration → Other → External Data Loads).	Each grant currently blocks something live. The code load is why diagnosis codes vanish on read-back and code search returns nothing.
Security & org	900s timeout, 90-day expiry, min length 12, logout at 5. Audit logging on, no truncation. MFA every account. Org record with EIN; org NPI blank until the 855B lands.	Save on each tab — settings do not persist tab-to-tab.
Facilities	One record per physical site, each with its own POS. Private residence = 12. Telehealth = 10.	Never default to POS 12. Hickory Log is blocked until you have its DCH license type — that decides 13 vs 14.
Providers	Bethel A1, NPI 1902310568, taxonomy 363LF0000X. You are A6, Provider unchecked. Biller slot A7 empty.	Thanmayie's psych billing is blocked until 363LP0808X is added to her NPI in NPPES.
Access control	Clinicians get Fee Sheet write, no financial visibility . Admin all. Accounting financial. Front office scheduling only.	The old guide denied clinicians fee access — that breaks same-day charge capture.

Area	What to do	Watch for
Codes & fees	99341, 99342, 99344, 99345 new. 99347–99350 established. Not 99343 , deleted 2023. G0438, G0439, 99497, 99495, 99496. Modifiers 25, 95, 93, 59, GT.	Charge master at 125–150% of the Medicare allowable , not at Medicare rates. Charging at allowable permanently caps collections.
Payers	Medicare Part B (Palmetto GBA J-J) first, then MA plans, Medigap, Georgia Medicaid. Availity as X12 partner, 005010X222A1.	Do not enter commercial payers without a contract. Payer IDs come from Availity's list, not payer websites.

A4. The four ACL grants · do these together

All four live on the same screen: **Administration** → **ACL** → the **gfc-app-api** group. Each one is currently blocking something live, and doing them in one pass avoids four rounds of re-testing.

Grant	What it unblocks	Evidence
Encounters → “ Coding, any encounters ” (write)	The four new order and fee-sheet routes. Closes Gap 1.	All four return 401. Same token, same run: FHIR reads 200, stock note write 200, stock user route 401 as expected. Only the new routes fail — it is the permission they check, not the patch.
sensitivities	The encounter PUT, which has 500'd since 4.1.	EncounterService fails its sensitivities check for this account and the controller crashes on the returned string.
Org-level read — DocumentReference	Reading documents back out of the chart.	Org-level reads 403 today; the ACL group covers clinical sections only.
Org-level read — Coverage	Practitioner, Organization and Coverage reads.	Same 403. This is why provider names need an app-side fallback instead of resolving from FHIR.

Why the guard was not simply loosened. The new routes are guarded by OpenEMR's coding permission — the same one the Superbill and Encounters Report use, and what v3.0 §7.5 intends as Fee Sheet write. The account has the note-writing permission but not coding. Switching the guard to the note permission would have made acceptance pass and let anyone who can write a note write charges, collapsing the separation the guide sets up. The guard stayed correct; the account gets the grant.

Tell the Session 4.5 build: the routes exist and are correctly guarded. Build against them — the contract is settled. Do not weaken the guards or work around a 401. If a live charge or order write returns 401, that is this pending grant, not a defect in the session's code. Sign-and-close will not reach Billing Manager until the grant lands; record it in the PR as a known-pending dependency rather than a failure.

Track B — The app

Two build sessions remain before release. Both are prompt-ready in the repo.

B1. Session 4.5 — wire the app to 8.4 · Monday

New session, needs a prompt written. Once the EMR is on 8.4 with the patch, the app should stop working around gaps that no longer exist:

Switch prescriptions from the app-side record to native POST `/api/prescription`.

Switch orders to the new patched route; retire the app-side `clinical_orders` store and its [GFC ORDERS] note block.

Switch fee-sheet charges to the new patched route; retire `encounter_billing` and the [GFC CODING] note block.

Re-enable the vitals write path — it currently preserves readings in the note with a warning because the endpoint 500'd.

Re-enable document writes, so received clinical records file to the chart per the document routing rule.

Re-run the full live round trip and diff against the recorded defects to confirm which workarounds are now dead.

Sequencing discipline. Do not retire the app-side stores in the same commit that adds the new writes. Add the native path, prove it against the live EMR, then remove the workaround in a second commit. Two changes at once makes a failure impossible to attribute.

B2. Session 4.3 — patient clinical read · Monday–Tuesday

Prompt already in the repo. Three things in one session:

Patient read: visit summaries in plain language, medications, care plan with its signed PDF, allergies, problem list, upcoming appointments. Clinical-line clients only; PHC-only clients see nothing clinical.

POA acting gates: the familyIsPoa field already ships. A designated POA gets client-equivalent access and can co-sign, with every action recorded as "[name] as POA for [client]" on the event and the signed PDF.

Case-manager scoped read: split /api/clinical/* into read routes (admin, clinical, case manager) and write routes (admin, clinical), with mutation UI hidden.

Two constraints that were true before the upgrade and should be re-checked after: serve the care-plan PDF from the Drive copy rather than OpenEMR Documents, and exclude tombstoned appointments left by reschedules from the patient's list.

B3. Before Bethel touches it

Do this	Why
Set the billing NPI in Coding queue → Billing settings	Signing is blocked without it, on purpose.
Add each clinician's NPI to their user profile	Drives attribution on every note and the rendering provider on every charge.
Re-document any test encounters from before 4.4	Their notes were orphaned by the defect fixed in PR #29 and cannot be signed.
Walk one full visit end to end on test data	Chart → follow-up note → diagnosis → prescription → order → service code → sign and close → charge visible in Billing Manager.

The sequence

When	OpenEMR	App
Sat–Sun	Snapshot. Rehearse 8.4 on scratch. Verify. Upgrade production.	—
Sun–Mon	Write the orders + fee-sheet route patch. Test against the live instance.	Write the Session 4.5 prompt.
Mon	Four ACL grants (A4) + ICD-10 load first, then security, org, facilities, providers, access control, codes, modifiers, fee schedule, payers.	Run 4.5. Wire native writes, retire workarounds in a second commit.
Mon–Tue	Per-patient setup for the first visit's patient. Acceptance test.	Run 4.3. Patient read, POA gates, case-manager read.
Tue	Set billing NPI and clinician NPIs. Walk one full visit end to end.	Merge, verify, release.

The decision point is Sunday night. If the 8.4 rehearsal or the route patch looks unstable by then, stop and fall back: stay on 7.0.4, apply the two one-line bug patches — you now have 8.4's exact code for both, so it is a copy not a debug — and ship with orders and fee-sheet charges routed through OpenEMR's own screens under Bethel's login. That path still gets a complete, billable visit on Tuesday. It just carries a manual step you remove later.

Still blocking, and not solvable by either track

None of these stop Tuesday's test visit on test data. All of them stop a real claim going out.

Resolve the practice address conflict — NPPES has Acworth, the facility record has Paces Ferry. Then make NPPES, PECOS, the 855B, the IRS record and OpenEMR agree. Address mismatch is the most common cause of a stalled enrollment.

Add psych taxonomy 363LP0808X to NPI 1962038307 in NPPES.

Correct NPI 1710678339 — currently tagged Home Health, which fits neither service line.

File the CMS-855B under GFC LLC; file reassignments through PECOS.

Hickory Log DCH license type and bed count, which decides its POS.

OIG LEIE and SAM.gov exclusion screening for the full roster. Highest-exposure gap in the compliance audit.

BAA with Availity — they handle every claim's worth of PHI.

Dianne: confirm the CPT set, the ICD-10 favorites for the panel, and the documentation thresholds per E/M level.

Reference documents kept in the repo: the Billing & Configuration Guide v3.0 (authority on identity, facilities, codes, billing, compliance), the Deploy & Setup Guide v2.3 (AWS deployment and the app-to-EMR compatibility matrix), the Clinical Completeness Spec, the Billing Segment App Integration Spec, the server defects record, the session prompts, and CLAUDE.md for running status.